NCCP 00731 · clinician draft

TCHP · subcutaneous trastuzumab

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · carboplatin combination

Active medicines: Docetaxel + Carboplatin + Trastuzumab + Pertuzumab

Emetogenic risk: High · carboplatin AUC ≥4

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1
Ondansetron	16 mg orally	Day 1 Individual dose / QT / hepatic review
Dexamethasone	12 mg orally	Day 1 Reconcile chemotherapy premedication steroids
Aprepitant	80 mg orally daily	Days 2–3
Metoclopramide	10 mg orally up to three times daily	Subsequent days: duration and use to be confirmed Individual contraindications, renal function and duration review
Dexamethasone	8 mg orally daily	Days 2–4 Carboplatin AUC ≥4 antiemetic row; reconcile total steroid exposure

Additional phase-specific support

- Docetaxel: dexamethasone 8 mg orally twice daily for 3 days starting the day before treatment; minimum 3 doses pre-treatment unless contraindicated. If oral doses were missed, any 20 mg IV replacement is a consultant decision.
- Reconcile docetaxel premedication with the antiemetic steroid course and interactions; issue one coherent steroid prescription, not duplicate courses.
- Consider primary G-CSF using the source and individual risk; product and timing follow the prescribed plan.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00731 v2 · p7 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-21 · SACTCheck v0.84.16 · Regimen-level drafts; prescriber and oncology-pharmacy verification required